

# Class 1 – Forearm Fracture: Patient Care Guide

A complete, easy-to-understand guide for patients who have a forearm fracture (a break in the radius and/or ulna, the two bones between the elbow and wrist). It covers causes, symptoms, diagnosis, treatment, healing, diet, exercises, warning signs, and frequently asked questions.

Medical disclaimer: This document is an educational reference created for a patient-support knowledge base and for use in a retrieval-augmented (RAG) AI assistant. It is NOT a substitute for professional medical advice, diagnosis, or treatment. Healing times, medicines, and exercises differ for every person. Always follow the instructions of your own doctor, orthopaedic surgeon, or physiotherapist. In an emergency, contact local emergency services immediately.

## 1. Introduction

A forearm fracture is a break in one or both of the two long bones of the forearm — the radius (on the thumb side) and the ulna (on the little-finger side). These bones run between the elbow and the wrist and work together to let you turn your palm up and down and to bend the elbow and wrist. A break in this area is also called a broken forearm.

Which bones are involved: the radius, the ulna, or both. When both bones break it is called a 'both-bone forearm fracture'. Special named patterns include the Monteggia fracture (ulna break with dislocation of the radius at the elbow) and the Galeazzi fracture (radius break with injury to the wrist joint).

Common causes: falling onto an outstretched hand, direct blows, sports injuries, road accidents, and twisting injuries. How common it is: forearm fractures are very common, especially in children and active adults, and are among the most frequent arm injuries seen in emergency rooms.

Who is at higher risk: children (whose bones are still growing), older adults with weak bones (osteoporosis), athletes in contact or wheel sports, and people in physically demanding or high-fall-risk jobs. Severity levels range from a simple hairline crack (mild) to a displaced or open both-bone fracture (severe).

## Types of Fractures Affecting This Bone

A forearm fracture can happen in several patterns. Common fracture types include:

- Hairline fracture (a very thin, small crack in the bone).
- Stress fracture (a tiny crack caused by repeated force or overuse).
- Greenstick fracture (the bone bends and cracks on one side; common in children).
- Closed fracture (the bone breaks but the skin stays intact).
- Open (compound) fracture (the broken bone breaks through the skin).
- Comminuted fracture (the bone breaks into three or more pieces).
- Spiral fracture (a break that twists around the bone, often from a twisting injury).
- Impacted fracture (the broken ends are driven into each other).
- Transverse fracture (a straight break across the bone).
- Oblique fracture (a slanted or angled break across the bone).

In children, greenstick and buckle (torus) fractures are especially common because young bones are softer and bend before they break.

## 2. Anatomy

Understanding the forearm helps you see why care and rehabilitation matter.

- Bones involved: the radius and ulna. They pivot around each other to turn the palm up (supination) and down (pronation).
- Nearby muscles: the forearm flexor muscles (front) bend the wrist and fingers, and the extensor muscles (back) straighten them; these move the hand and grip.
- Tendons: strong cords that connect these muscles to the wrist and finger bones; injury or swelling here can limit hand movement.
- Ligaments: tough bands that hold the radius and ulna together (including the interosseous membrane between the bones) and stabilise the elbow and wrist joints.
- Blood vessels: the radial and ulnar arteries run through the forearm to supply the hand; injury can affect blood flow and healing.
- Nerves: the median, ulnar, and radial nerves pass through the forearm and control hand movement and feeling; pressure or injury can cause numbness or weakness.

Because the two bones must glide smoothly to rotate the forearm, a break that heals in a poor position can limit the ability to turn the palm. Damage to nerves or vessels can also slow recovery, which is why doctors check hand movement, sensation, and pulse carefully.

## 3. Common Causes

Most fractures happen when a force is stronger than the bone can handle. The main causes are:

- Falls: slipping, tripping, or falling onto an outstretched hand or arm — the most common cause overall.
- Road traffic accidents: high-speed impacts from car, motorbike, or cycle crashes.
- Sports injuries: contact sports, skating, cycling, gymnastics, and falls during play.
- Workplace accidents: falls from height, machinery injuries, or heavy objects.
- Direct trauma: a hard blow or hit directly on the bone.
- Twisting injuries: the limb turns while the hand or body stays fixed.
- Heavy object injuries: something heavy falling onto the limb.
- Osteoporosis: weak, thin bones that break easily, especially in older adults.
- Bone tumors: growths that weaken the bone (pathological fracture).
- Repetitive stress: repeated overuse leading to stress fractures over time.

For the forearm specifically, a fall onto an outstretched hand (often called a FOOSH injury) and direct impact during sports or accidents are the leading causes.

## 4. Symptoms

Common signs and symptoms of a forearm fracture include:

- Pain in the forearm, made worse by movement or touch.
- Swelling around the injured area.

- Bruising or discolouration of the skin.
- A visible deformity, bend, or 'crooked' look to the forearm.
- Difficulty or inability to turn the palm up or down.
- Numbness or tingling in the hand or fingers.
- Reduced grip strength or trouble holding objects.
- A snapping or cracking sound at the time of injury.
- In an open fracture, bone may be seen through a wound.

Emergency warning signs: bone sticking through the skin, a cold or blue hand, spreading numbness, uncontrolled bleeding, or severe worsening pain. Seek emergency care immediately.

## 5. Diagnosis

Doctors confirm a fracture using a physical check and imaging tests. Each test gives different information.

### Physical Examination and Medical History

The doctor asks how the injury happened and checks for pain, swelling, bruising, deformity, tenderness, movement, pulse, skin colour, and sensation. Telling the doctor about past fractures, osteoporosis, medicines, and other illnesses helps plan the right treatment.

### Imaging Tests

- X-ray: the first and main test. It shows most fractures, the exact location, and whether the bone has moved out of place (displacement).
- CT scan: gives detailed 3-D images. Used for complex breaks, joint involvement, or when the X-ray is unclear, to plan surgery.
- MRI: shows soft tissues, bone bruising, and hairline or stress fractures that may not appear on an X-ray. Also checks nearby ligaments, tendons, and nerves.
- Ultrasound: sometimes used, especially in children, to look at bone surfaces and soft-tissue swelling without radiation.
- Bone scan: can detect very small stress fractures or hidden fractures that other tests miss, by showing areas of increased bone activity.

For a forearm fracture, X-rays of the whole forearm — including the elbow and wrist joints — are essential to rule out associated dislocations (Monteggia and Galeazzi patterns). A CT scan may be added for complex or joint-involving breaks.

## 6. Treatment Options

Treatment depends on the type of break, how far the bone has moved, and your age and health. It falls into non-surgical and surgical options.

### Non-Surgical Treatment

Many stable forearm fractures, especially in children, heal well without surgery.

- Cast: a plaster or fibreglass cast (often an above-elbow cast) holds the bones still while they heal; commonly worn for several weeks.
- Splint: used first, especially when there is a lot of swelling, before a full cast.
- Brace: a removable functional brace may be used in later healing for some fractures.
- Sling: supports the arm, keeps it raised, and reduces pain.
- Ice therapy: reduces pain and swelling in the first days.
- Elevation: keeping the forearm raised lowers swelling.
- Pain medications: paracetamol for pain relief.
- Anti-inflammatory medicines: such as ibuprofen, if approved by your doctor.

When recommended: non-surgical care suits stable fractures where the bone is in a good position. Advantages: no surgery, lower infection risk, and no anaesthesia. Limitations: needs a well-set position, the cast can cause stiffness, and it may not hold badly displaced or unstable fractures.

## **Surgical Treatment**

Surgery is used when the bone is badly out of place, broken into many pieces, involves a joint, is an open (compound) fracture, or fails to heal with a cast. The aim is to put the pieces back in the correct position and hold them still while they heal.

- ORIF (Open Reduction Internal Fixation): the surgeon opens the skin, sets the bone in the right position (reduction), and fixes it with metal implants.
- Plates: flat metal supports screwed along the bone to hold the pieces together.
- Screws: hold fragments together, alone or with a plate.
- Pins (K-wires): thin wires that hold small pieces, often used in fingers and in children; usually removed later.
- Rods (intramedullary nails): metal rods placed inside the hollow centre of a long bone to support it.
- External fixator: a frame outside the body connected to the bone by pins; used for severe, open, or badly contaminated fractures.
- Bone graft: extra bone (from your own body or a donor) added to help healing when there is bone loss or slow healing.

Indications (when surgery is needed): both-bone forearm fractures in adults, displaced fractures, unstable or open fractures, Monteggia and Galeazzi injuries, and fractures that cannot be held in a good position by a cast. Adults with both-bone breaks usually need a plate-and-screw fixation.

## **Recovery Expectations and Risks After Surgery**

- Recovery: a protective cast, splint, or sling is often used after surgery, followed by physiotherapy. Implants usually stay in place unless they cause problems.
- Risks: infection, bleeding, nerve or blood vessel injury, implant irritation or loosening, stiffness, blood clots, and reaction to anaesthesia.
- Most people regain good function, but full recovery takes weeks to months and depends on the injury and following rehabilitation.

## 7. Healing Timeline

Healing time varies by age, general health, fracture type, and how well instructions are followed. The table gives typical ranges.

| Group        | Typical healing / recovery                                                         |
|--------------|------------------------------------------------------------------------------------|
| Children     | Often 4–6 weeks to unite; remodelling can correct minor angles over months.        |
| Adults       | About 8–12 weeks for bone union; longer for both-bone or surgical fractures.       |
| Older adults | 12 weeks or more; healing may be slower with osteoporosis.                         |
| Athletes     | Bone union in 8–12 weeks, but return to sport often 3–4 months with full strength. |

Stages of healing: initial healing (inflammation and soft callus) in the first weeks, bone union (hard callus bridges the break), and functional recovery (regaining strength and movement). Turning the palm fully up and down is often the last movement to return.

- Return to work: desk work in 1–2 weeks; heavy manual work often 10–12 weeks or after clearance.
- Return to sports: non-contact around 8–10 weeks; contact sports usually 3–4 months.
- Full recovery: 3–6 months for full strength and rotation in most adults.

Factors that delay healing: smoking, alcohol, poor nutrition, diabetes, osteoporosis, infection, poor blood supply, older age, certain medicines (such as steroids), and not resting the injured limb.

## 8. Recovery Stages

Knowing what to expect at each stage helps you feel reassured. Your own timeline may differ slightly.

| Stage    | What usually happens                                                           |
|----------|--------------------------------------------------------------------------------|
| Week 1   | Most pain and swelling; arm kept elevated in cast/splint; move fingers gently. |
| Week 2   | Pain settling; swelling reducing; keep using fingers, shoulder, and hand.      |
| Week 3–4 | Cast continues; bone starting to knit; stiffness in wrist/fingers is common.   |
| Week 5–8 | Bone uniting; cast may come off toward the end; gentle movement begins.        |
| Month 3  | Bone usually united; physiotherapy to regain rotation and strength.            |
| Month 6  | Most people near full strength and movement; heavy tasks resumed.              |

## 9. Daily Care Instructions

- Keep the cast dry: cover it with a plastic bag while bathing; a wet cast weakens and can irritate the skin.
- Sleeping position: keep the arm supported on a pillow, slightly raised, to reduce swelling; avoid lying on the injured arm.
- Bathing: use a waterproof cover or sponge-bathe; dry the edges of the cast if damp.
- Clothing: wear loose sleeves or button shirts; dress the injured arm first and undress it last.

- Walking: normal walking is fine; keep the arm in the sling for comfort and support.
- Driving: avoid driving while in an arm cast or when unable to control the wheel safely; ask your doctor when it is safe.
- Returning to office: light desk work is often possible within days to a couple of weeks if pain allows.
- Returning to school: children can usually attend once comfortable, avoiding sports and rough play.
- Household activities: avoid lifting, carrying, and using the injured arm for heavy tasks; use the other hand.

## 10. Pain Management

Some pain is normal after a fracture, especially in the first days. These safe, simple methods help control it.

- Ice packs: wrap ice in a cloth and apply for 15–20 minutes several times a day to reduce pain and swelling (never put ice directly on the skin).
- Elevation: keep the injured limb raised above the level of the heart to reduce throbbing and swelling.
- Rest: avoid using the injured limb for gripping, lifting, or bearing weight.
- Painkillers: paracetamol (acetaminophen) is commonly used. Anti-inflammatory medicines such as ibuprofen may be advised. Take only the dose your doctor approves.
- Safe positioning: support the limb on a pillow and avoid letting it hang down.
- Relaxation techniques: slow breathing, distraction, and gentle music can reduce the sensation of pain.

When pain is abnormal: pain that suddenly becomes severe, is not relieved by medicine, comes with numbness, a cold or blue hand, or a very tight cast may signal a serious problem such as compartment syndrome or poor blood supply. Seek medical help urgently.

## 11. Diet for Bone Healing

Good nutrition speeds up bone healing. The nutrients below all play a role. Try to eat a balanced diet that includes them every day, and follow any special diet advice from your doctor or dietitian.

### Key Nutrients for Bone Healing

| Nutrient  | Why it helps & daily intake                                                   | Vegetarian sources                                                           | Non-vegetarian sources                                  |
|-----------|-------------------------------------------------------------------------------|------------------------------------------------------------------------------|---------------------------------------------------------|
| Calcium   | Main mineral that builds and repairs bone. About 1000–1200 mg/day for adults. | Milk, curd, paneer, cheese, ragi, tofu, green leafy vegetables, sesame seeds | Small fish eaten with bones (sardines), dairy           |
| Vitamin D | Helps the body absorb calcium. About 600–800 IU/day (more if deficient).      | Sunlight, fortified milk and cereals, mushrooms                              | Egg yolk, fatty fish (salmon, mackerel), fish liver oil |
| Protein   | Builds the collagen framework of bone. About 1.0–1.5 g per kg body weight.    | Dal, beans, chickpeas, rajma, soya, paneer, tofu, nuts                       | Eggs, chicken, fish, lean meat                          |

| Nutrient     | Why it helps & daily intake                                        | Vegetarian sources                                       | Non-vegetarian sources                  |
|--------------|--------------------------------------------------------------------|----------------------------------------------------------|-----------------------------------------|
| Vitamin C    | Needed to make collagen for bone repair. About 65–90 mg/day.       | Oranges, lemon, amla, guava, capsicum, tomato, broccoli  | (Mainly from plant foods)               |
| Vitamin K    | Helps bind calcium into bone. About 90–120 mcg/day.                | Spinach, kale, broccoli, cabbage, green leafy vegetables | Eggs, some dairy                        |
| Magnesium    | Supports bone structure and vitamin D use. About 310–420 mg/day.   | Nuts, seeds, whole grains, bananas, dark leafy greens    | Fish                                    |
| Phosphorus   | Works with calcium to harden bone. About 700 mg/day.               | Dairy, beans, lentils, whole grains, nuts                | Meat, fish, poultry, eggs               |
| Zinc         | Helps bone cells grow and repair. About 8–11 mg/day.               | Whole grains, legumes, seeds, nuts                       | Meat, shellfish, eggs                   |
| Omega-3 fats | Reduce inflammation and support healing.                           | Flaxseed, chia seeds, walnuts, soybean oil               | Fatty fish (salmon, mackerel, sardines) |
| Collagen     | Forms the flexible base of bone. Supported by protein + vitamin C. | Body makes it from protein-rich plant foods + vitamin C  | Bone broth, chicken, fish, eggs         |

Drink enough water and, if your doctor recommends, take calcium and vitamin D supplements. Do not take high-dose supplements on your own without medical advice.

## 12. Foods to Avoid

Some foods and habits slow bone healing by reducing calcium absorption, increasing inflammation, or harming blood supply. Limit or avoid the following while your fracture heals.

- Alcohol: slows new bone formation and reduces calcium and vitamin D use. Best avoided completely during healing.
- Smoking and tobacco: reduce blood flow to the bone and are a leading cause of delayed healing and nonunion. Stopping greatly improves healing.
- Sugary foods: too much sugar can increase inflammation and displace healthy foods.
- Soft drinks and colas: high phosphoric acid and sugar may reduce calcium balance.
- Excess caffeine: large amounts of coffee, tea, or energy drinks can reduce calcium absorption; keep intake moderate.
- Processed and packaged food: often high in salt and low in nutrients.
- High sodium (salt) intake: makes the body lose more calcium in urine.
- Junk and deep-fried food: high in unhealthy fat and low in bone-building nutrients.

In short, a fresh, home-cooked, balanced diet with less salt, sugar, alcohol, and no smoking gives your bone the best chance to heal quickly.

## 13. Physical Therapy and Rehabilitation Exercises

Physiotherapy restores movement, strength, and function. Start only when your doctor allows, and stop any exercise that causes sharp or increasing pain. Below are typical stages — your therapist will tailor them to you.

### Early Stage

- Finger, thumb, and shoulder movements while in the cast — purpose: prevent stiffness and swelling. Repetitions: gentle bends, 10 times, several times a day. Precaution: keep the wrist and forearm still.
- Gentle fist opening and closing — purpose: keep tendons gliding and reduce swelling.

### Middle Stage

- Wrist bending up and down (after cast removal) — purpose: restore wrist movement. Repetitions: 10 each way, 2–3 times daily. Precaution: no pain past a gentle stretch.
- Forearm rotation (turning palm up and down) — purpose: regain the key forearm movement. Repetitions: 10 slow turns. Precaution: move within a comfortable range.
- Elbow bending and straightening — purpose: reduce elbow stiffness.

### Advanced Stage

- Grip strengthening with a soft ball or putty — purpose: rebuild hand strength. Repetitions: squeeze 10–15 times, 2–3 sets. Precaution: start light.
- Resistance-band wrist and forearm exercises — purpose: build strength and endurance.
- Gradual return to lifting and sport-specific drills — purpose: safe return to full activity. Precaution: increase load slowly and stop if pain returns.

General exercise precautions: move slowly, never push through sharp pain, stop if you feel numbness or dizziness, and follow the exact repetitions your therapist advises. When to stop: stop and call your therapist if pain worsens, swelling increases, or movement suddenly decreases.

## 14. Activities to Avoid During Healing

Avoiding certain activities protects the healing bone and implants.

| Activity                                         | How long to avoid (typical)                    |
|--------------------------------------------------|------------------------------------------------|
| Heavy lifting with the injured arm               | Until healed and cleared — often 10–12 weeks   |
| Push-ups, pull-ups, weight lifting               | About 3 months or until cleared                |
| Contact sports (cricket, football, martial arts) | About 3–4 months                               |
| Carrying heavy bags on that arm                  | Until full strength returns (2–3 months)       |
| Forceful twisting of the forearm                 | Until rotation is pain-free (about 8–12 weeks) |
| Driving with the arm in a cast                   | Until the cast is off and control is safe      |

## 15. Possible Complications

Most fractures heal well, but knowing the possible problems helps you spot them early. Report any of these to your doctor.

- Delayed union: the bone heals more slowly than expected. Prevented by good nutrition, not smoking, and following restrictions.
- Nonunion: the bone fails to join. Signs are ongoing pain and movement at the break site; may need surgery or a bone graft.
- Malunion: the bone heals in the wrong position, causing deformity or reduced function; sometimes needs correction.
- Infection: mainly with open fractures or after surgery. Signs are fever, redness, warmth, swelling, pus, or bad smell.
- Stiffness: nearby joints become stiff from being kept still; prevented by physiotherapy and gentle movement as advised.
- Nerve injury: causes numbness, tingling, or weakness; often improves but should be checked.
- Blood vessel injury: rare but serious; causes a cold, pale, or blue limb and needs emergency care.
- Chronic (long-lasting) pain: pain that continues after healing; managed with therapy and pain specialists.
- Arthritis: joint fractures can lead to wear-and-tear arthritis years later.
- Reduced mobility: lasting loss of full movement, reduced by good rehabilitation.
- Complex Regional Pain Syndrome (CRPS): an uncommon condition with severe pain, swelling, skin colour and temperature changes; early treatment gives the best result.

## 16. Red Flag Symptoms — Go to Hospital Immediately

Get emergency medical help right away if you notice any of the following. These can mean the blood supply, nerves, or the healing is in danger.

- Severe or rapidly increasing swelling.
- Fingers (or toes) turning blue, grey, or very pale.
- The hand or limb feels cold to touch.
- Numbness or loss of feeling that does not go away.
- Fever or chills (a sign of infection).
- Bleeding that will not stop.
- Severe pain that is not relieved by rest, elevation, or prescribed medicine.
- The cast becomes too tight or the limb feels squeezed.
- The cast breaks, cracks, or becomes loose.
- Pus, discharge, or fluid coming from under the cast or a wound.
- A bad smell coming from the cast or wound.
- Bone is visible through the skin (open fracture).
- Difficulty breathing, chest pain, or coughing after the injury (a possible blood clot or fat embolism — a rare but serious emergency).

Compartment syndrome warning: severe deep pain, tightness, numbness, and pain when the fingers are gently stretched can signal dangerous pressure build-up. This is an emergency — do not wait.

## 17. Follow-Up Schedule

Regular follow-up lets the doctor check that the bone is healing in the right position. A typical schedule is below.

| Visit   | What the doctor usually checks                                                    |
|---------|-----------------------------------------------------------------------------------|
| Week 1  | Check swelling, cast fit, blood flow, and position on a repeat X-ray.             |
| Week 2  | Confirm the bone has not shifted; adjust splint/cast if needed.                   |
| Week 4  | X-ray to check early bone healing (callus); assess pain and movement.             |
| Week 6  | Check for bone union; the cast may be removed or changed to a brace.              |
| Week 12 | Confirm full union; review strength, rotation, and plan for return to sport/work. |

## 18. Returning to Normal Life

These are general guides for when people usually return to activities. Always get your doctor's clearance first.

| Activity           | Typical time to return                                                 |
|--------------------|------------------------------------------------------------------------|
| School             | A few days to 1–2 weeks (no sports/rough play)                         |
| Office (desk job)  | A few days to 2 weeks                                                  |
| Driving            | After the cast is off and control is safe — often 6–8 weeks            |
| Gym (upper body)   | About 3 months, building up gradually                                  |
| Swimming           | After the cast is off and the wound/skin has healed — about 8–10 weeks |
| Cycling            | Light cycling around 8–10 weeks; avoid falls risk                      |
| Running            | Often 6–8 weeks (arm must be pain-free and stable)                     |
| Cricket / Football | About 3–4 months with clearance                                        |
| Weight lifting     | About 3–4 months, increasing load slowly                               |

## 19. Prevention of Future Fractures

After one fracture, it is worth protecting your bones for the future. These steps lower the chance of breaking a bone again.

- Bone-strengthening exercises: regular resistance and muscle-strengthening activity keeps bones dense.
- Weight-bearing exercise: walking, jogging, dancing, and stair climbing build bone strength.
- Balance training: yoga, tai chi, and balance drills reduce falls, especially in older adults.
- Vitamin D: from sunlight, diet, or supplements as advised, to help absorb calcium.
- Calcium intake: eat enough calcium-rich foods every day.
- Protective equipment: wrist guards, elbow pads, and helmets during sports and skating.
- Safe workplace habits: follow safety rules, use guards, and prevent falls from height.

- Sports safety: warm up, use correct technique, and wear the right gear.
- Fall prevention: good footwear, adequate lighting, and vision checks.
- Home safety tips: remove loose rugs, add grab bars, keep floors dry and clutter-free.
- Osteoporosis screening: a bone density (DEXA) scan for older adults and high-risk people, with treatment if bones are weak.

## 20. Frequently Asked Questions (FAQ)

### Q1. Can I move my fingers?

Yes. Gently moving your fingers, thumb, and shoulder while in the cast is encouraged. It keeps them from getting stiff and helps reduce swelling. Avoid moving the wrist and forearm inside the cast.

### Q2. Can I sleep on the injured arm?

It is best not to sleep on the injured arm. Rest it on a pillow, slightly raised, to keep it comfortable and reduce swelling.

### Q3. Can I remove the cast or splint myself?

No. Never remove or cut your cast or splint yourself. It must stay on until your doctor decides the bone has healed enough.

### Q4. When can I drive?

Usually only after the cast is removed and you can safely control the steering wheel and gears — often around 6–8 weeks. Ask your doctor first.

### Q5. Can I exercise?

You can do gentle finger, shoulder, and light lower-body exercise early on. Wait for your doctor's advice before forearm, wrist, or upper-body workouts.

### Q6. Can I lift weights?

Not until the bone has healed and you are cleared — often around 3–4 months, starting light and increasing slowly.

### Q7. Is swelling normal?

Some swelling is normal in the first days to weeks. Keep the arm raised and use ice. Sudden, severe, or increasing swelling needs urgent medical review.

### Q8. Is itching inside the cast normal?

Yes, mild itching is common. Do not push objects inside the cast to scratch, as this can injure the skin and cause infection. Cool air from a hair dryer on a low setting can help.

### Q9. Can I travel?

Short travel is usually fine once pain is controlled. Keep the arm raised and supported. Ask your doctor before long journeys.

### Q10. Can I fly?

Air travel is generally allowed, but tell your doctor. On long flights soon after a new cast, some doctors advise a split cast because swelling can increase.

### **Q11. Can I get the cast wet?**

No. A standard plaster or fibreglass cast must stay dry. Use a waterproof cover for bathing. A wet cast can weaken and irritate the skin.

### **Q12. How long should I wear the sling?**

A sling is usually worn for comfort in the first 1–3 weeks, or as your doctor advises. Remove it for gentle movement if allowed.

### **Q13. When should I see a doctor urgently?**

If you have severe pain, a cold or blue hand, spreading numbness, a very tight or broken cast, fever, or discharge, seek care at once.

### **Q14. How long will the whole forearm fracture take to heal?**

Bone union usually takes about 8–12 weeks in adults and 4–6 weeks in children, with full strength over 3–6 months.

### **Q15. Will I need surgery?**

Not always. Many fractures heal in a cast. Surgery is more likely for both-bone breaks in adults, displaced, unstable, or open fractures.

### **Q16. Do the metal plates or screws need to be removed?**

Usually they can stay for life. They are removed only if they cause pain, irritation, or infection.

### **Q17. Why can't I turn my palm properly?**

The two forearm bones must glide to rotate the palm. Stiffness after a fracture is common and usually improves with physiotherapy.

### **Q18. Can I type or use a phone?**

You can use your fingers gently for light typing or texting if it does not hurt and the wrist stays supported.

### **Q19. Is some pain normal weeks after the injury?**

Mild aching during healing is normal. Sharp, worsening, or new pain should be checked by your doctor.

### **Q20. Can children's forearm fractures straighten on their own?**

Yes. Because children's bones remodel as they grow, minor bends often correct themselves over months.

### **Q21. What food helps my bone heal faster?**

A balanced diet rich in calcium, vitamin D, protein, and vitamin C helps. Avoid smoking and alcohol, which slow healing.

### **Q22. Can I go to work?**

Desk jobs are often possible within days to two weeks. Heavy manual work usually needs 10–12 weeks or your doctor's clearance.

**Q23. Will my arm be weak after the cast comes off?**

Yes, temporary weakness and stiffness are normal after weeks in a cast. Physiotherapy restores strength and movement.

**Q24. Can I play sports again?**

Yes, most people return to sport. Non-contact around 8–10 weeks and contact sports around 3–4 months, after clearance.

**Q25. How can I prevent another forearm fracture?**

Strengthen bones with exercise, get enough calcium and vitamin D, prevent falls, wear protective gear in sport, and get screened for osteoporosis if you are at risk.

**Q26. My cast feels too tight — what should I do?**

A cast that becomes very tight, painful, or causes tingling or a cold hand needs urgent review; do not wait, contact your doctor.

**21. Metadata (for Retrieval)**

Structured tags to help the AI assistant retrieve this document accurately.

| Field                   | Value                                                                                                            |
|-------------------------|------------------------------------------------------------------------------------------------------------------|
| Fracture Name           | Forearm Fracture                                                                                                 |
| Bones Involved          | Radius, Ulna                                                                                                     |
| Body Region             | Upper limb (forearm, between elbow and wrist)                                                                    |
| Common Causes           | Falls (FOOSH), direct trauma, sports, road accidents, twisting, osteoporosis                                     |
| Symptoms                | Pain, swelling, bruising, deformity, limited rotation, numbness, weak grip                                       |
| Treatment Options       | Cast, splint, sling; ORIF with plates/screws, rods, external fixator, bone graft                                 |
| Healing Duration        | Children 4–6 weeks; adults 8–12 weeks; full strength 3–6 months                                                  |
| Rehabilitation Duration | 6–16 weeks of physiotherapy depending on severity                                                                |
| Emergency Signs         | Open bone, cold/blue hand, spreading numbness, uncontrolled bleeding, compartment syndrome                       |
| Keywords                | forearm fracture, broken forearm, radius fracture, ulna fracture, both-bone fracture, Monteggia, Galeazzi, FOOSH |
| Synonyms                | Broken forearm, cracked forearm bone, radius/ulna break, lower arm fracture                                      |
| Related Injuries        | Wrist fracture, elbow fracture, distal radius fracture, humerus fracture                                         |